

Riverside Medical Centre

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Annual Health Check — Laboratory & Clinical Report

Patient: James Whitfield **DOB:** 05 Jan 1964 (62 y) · **Sex:** M · **MRN:** TEST-0003
Visit date: 04 Jun 2026 · **Ordering physician:** Dr. R. Ferran · **Reason:** Fatigue, polyuria, blurred vision × 6 weeks; smoker 30 pack-years

VITAL SIGNS

BP 162/98 mmHg (hypertensive) · HR 92 bpm · BMI 31.4 kg/m² (obese) · Temp 36.8 °C · SpO₂ 96%

COMPLETE BLOOD COUNT

Test	Result	Units	Reference range	Flag
Haemoglobin	11.2	g/dL	13.0 – 17.0	LOW
White cell count	12.4	10 ³ /μL	4.0 – 11.0	HIGH
Platelets	190	10 ³ /μL	150 – 400	

METABOLIC PANEL

Test	Result	Units	Reference range	Flag
Fasting glucose	248	mg/dL	70 – 99	HIGH
HbA1c	9.6	%	< 5.7	HIGH
Creatinine	1.8	mg/dL	0.7 – 1.3	HIGH
eGFR	42	mL/min/1.73m ²	> 60	LOW
ALT	74	U/L	7 – 56	HIGH

LIPID PANEL

Test	Result	Units	Reference range	Flag
Total cholesterol	262	mg/dL	< 200	HIGH
LDL cholesterol	176	mg/dL	< 130	HIGH
HDL cholesterol	32	mg/dL	> 40	LOW
Triglycerides	284	mg/dL	< 150	HIGH

URINALYSIS

Test	Result	Units	Reference range	Flag
Protein	2+		Negative	HIGH
Glucose	3+		Negative	HIGH

CLINICAL IMPRESSION

Markedly elevated glucose and HbA1c consistent with uncontrolled type 2 diabetes mellitus. Reduced eGFR with proteinuria suggesting stage 3 chronic kidney disease, likely diabetic nephropathy. Mild anaemia and elevated ALT requiring work-up. Uncontrolled hypertension and dyslipidaemia; high cardiovascular risk profile.

PLAN / RECOMMENDATIONS

Urgent: commence glucose-lowering therapy and antihypertensive treatment per endocrinology review (appointment within 2 weeks). Renal ultrasound and repeat renal function in 2 weeks. Iron studies and liver screen ordered. Statin initiated. Smoking-cessation referral. Diabetic retinopathy screening booked. Safety-netting advice given; return immediately if vomiting, drowsiness, or chest pain.

This document contains entirely fictional patient data generated for testing document-processing software. It is not medical advice and does not describe a real person.